Clinical Study Summary Report (CSSR) Document Status: Draft | **Version:** 0.1 | **Date:** 03-Apr-2026

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1. Administrative & Study Identification

Full Study Title: Iptacopan in Patients With ANCA Associated Vasculitis **Short Title/Acronym:** Iptacopan AAV Phase 2 Study **Protocol Number:** CLNP023R12201 **NCT Number:** NCT06388941 **Trial Status:** Recruiting **Sponsor Name:** Novartis **Investigational Product:** Iptacopan (LNP023) - oral proximal complement factor B inhibitor **Phase of Development:** Phase II **Medical Monitor:** Novartis Clinical Operations Lead

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the effect of iptacopan in achieving sustained remission compared to standard of care (SOC). **Secondary Objectives:** To assess the effect of iptacopan on B cell counts, total IgG levels, complete remission at week 24, disease relapses, evolution of renal function (eGFR) and proteinuria, glucocorticoid (GC) side effects, patients' immune status, and quality of life (QoL).

2.2 Background & Rationale ANCA-associated vasculitis (AAV) is a group of rare multi-system autoimmune diseases characterized by inflammation and damage to mainly small blood vessels. The alternative pathway (AP) of the complement system plays a central role in the pathogenesis of AAV. There are limited approved therapies, and an unmet clinical need remains for a therapy to induce persistent remission in patients with AAV while reducing glucocorticoid (GC) treatment and avoiding long-term B cell depletion. Iptacopan selectively inhibits the AP and may improve outcomes for these patients.

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind (during the 24-week Induction period) and Open-label (during the 24-week Maintenance period) **Randomization:** Randomized

3.2 Planned Enrollment & Duration Target N: 78 participants **Number of Sites:** Multiple global sites (e.g., across 9 European countries and the US) **Estimated Study Start:** Early 2024 **Estimated Primary Completion:** Late 2026

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 to ≤ 100 years at the time of screening. 2. Newly diagnosed or relapsed Granulomatosis with Polyangiitis (GPA) and Microscopic Polyangiitis (MPA) (according to the 2022 ACR/EULAR classification criteria for GPA and MPA) requiring treatment with rituximab (RTX) and glucocorticoids (GC) as per investigator's judgement. 3. Birmingham Vasculitis Activity Score (BVAS) assessment with ≥ 1 major item, or ≥ 3 minor items, or ≥ 2 renal items at Screening. 4. Positive antibody test for anti-proteinase 3 (PR3) or anti-myeloperoxidase (MPO) antibodies at Screening or with history of documented evidence of a positive antibody test.

4.2 Exclusion Criteria 1. Other systemic disease which constitutes the primary illness, including but not limited to: eosinophilic granulomatosis with polyangiitis (EGPA), moderate to severe systemic lupus erythematosus, IgA vasculitis (Purpura Schönlein-Henoch), rheumatoid vasculitis, Sjögren's syndrome, anti-glomerular basement membrane (GBM) disease, cryoglobulinemic vasculitis, autoimmune hemolytic anemia, autoimmune lymphoproliferative syndrome or mixed connective tissue disease. 2. Alveolar hemorrhage requiring invasive pulmonary ventilation support at Screening. 3. Severe kidney disease defined as estimated glomerular filtration rate (eGFR) < 15 mL/min/1.73m2, or kidney failure defined as receiving renal replacement therapy such as hemo(dia)filtration, hemo-/peritoneal dialysis, or having received a kidney transplant. 4. Received plasma exchange/-pheresis within 12 weeks prior to Screening.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Iptacopan 200mg twice daily or matching placebo. **Route of Administration:** Oral (hard gelatin capsules). **Duration of Treatment:** 48 weeks total (24-week double-blind induction period followed by a 24-week open-label maintenance period).

5.2 Concomitant & Prohibited Therapy Permitted: Standard of care therapies including Rituximab (RTX) induction and rapid, defined glucocorticoid (GC) tapering. **Prohibited:** Use of plasma exchange/-pheresis or prohibited medications that cannot be stopped.

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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Weeks -4 to 0	Informed Consent, BVAS assessment, Medical History, Antibody Labs
Baseline	Day 0	Randomization, First Dose, Induction Therapy Start
Interim	Week 24	Complete remission assessment, End of double-

blind Induction phase, Start of Open-label Maintenance | | **End of Study** | Week 48 | Final evaluation of sustained remission, GC dose check, Transition to safety follow-up |

(Note: A 4-week safety follow-up extends the trial to an End of Study visit at Week 52)

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Sustained remission through Week 48 defined as complete remission at Week 24 without major relapse up to Week 48. **Measurement Method:** Clinical evaluation utilizing the BVAS tool and documentation of daily glucocorticoid dosing regimens to assess the effect of iptacopan compared to standard of care (SOC).

7.2 Secondary & Exploratory Endpoints 1. Complete remission at week 24. 2. B cell counts. 3. Total IgG levels. 4. Effect of iptacopan on disease relapses and QoL. 5. Evolution of renal function (eGFR) and proteinuria. 6. Evaluation of GC side effects and patients' immune status.

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via clinical assessments, blood, and urine tests at protocol-defined visits. **Serious Adverse Events (SAEs):** Monitoring for severe infections, disease relapses, and adverse reactions, primarily driven by immunosuppression mechanisms. **Data Monitoring Committee (DMC):** Internal/External independent committee details per standard Novartis Phase 2 trial protocols.

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy evaluating sustained remission. Safety Set for evaluating treatment-emergent adverse events. **Sample Size Justification:** Target sample of 78 subjects powered to detect meaningful statistical superiority in achieving sustained remission over standard of care. **Handling of Missing Data:** Methods appropriate for Phase 2 longitudinal modeling (e.g., Multiple Imputation or LOCF) as defined in the clinical study protocol.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular onsite and remote monitoring for protocol compliance, BVAS grading accuracy, and safety reviews. **Audit Trail:** Robust eCRF systems documenting the rapid GC tapering schedule and RTX administration logs.

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11. Study Identifiers & Governance

Primary ID: CLNP023R12201 **Registry Number:** NCT06388941 **Sponsor/ Lead Organization:** Novartis **Study Title:** Iptacopan in Patients With ANCA Associated Vasculitis **Official Regulatory Title:** A Randomized, Controlled Study to Evaluate LNP023 (Iptacopan) in Patients With Active ANCA-associated Vasculitis

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12. Clinical Framework (AAV Specific)

Primary Condition: Anti-Neutrophil Cytoplasm Antibodies (ANCA) Associated Vasculitis / Granulomatosis with Polyangiitis (GPA) and Microscopic Polyangiitis (MPA) **Diagnosis Threshold:** Positive anti-PR3 or anti-MPO antibodies and BVAS with ≥ 1 major item or equivalent severity **Medical Methodology:** Standard of Care (Rituximab + Glucocorticoids) + Proximal Complement Inhibitor **Optimization Target:** Sustained remission defined as BVAS = 0 and GC dose tapering to ≤ 5 mg/day

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13. Study Procedures & Methodology

Management Pathway: AAV Remission Induction and Maintenance Therapy **Education Protocol (HCP):** BVAS scoring calibration and protocol-defined rapid GC tapering schedules **Education Protocol (Patient):** Self-monitoring of AAV symptoms, infection risk awareness, and medication adherence guidelines **Quality Audit Mechanism:** Routine assessment of renal function parameters (eGFR, hematuria) and sustained remission markers

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14. Observation & Follow-Up Schedule

Total Duration: 48 treatment weeks + 4 weeks safety follow-up **Onsite Visit Cadence:** Regular monitoring during induction; distinct checkpoints at Baseline, Week 24, Week 48, and Week 52 **Remote Monitoring Frequency:** Intermittent monitoring for GC dose adjustments per protocol **Checklist Review Interval:** Comprehensive evaluations at the end of Induction (Week 24) and End of Treatment (Week 48)

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15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Name]	____	[DD-MMM-YYYY]
Clinical Operations Lead	[Name]	___	[DD-MMM-YYYY]
Lead Statistician	[Name]	_____	[DD-MMM-YYYY]